

Value Based Program - Clinical Quality Metric Specifications 2017 Program Year

ADULT PROGRAM METRICS	
Metric Identifier	
	Prevention and Screening
BCS	Breast Cancer Screening
COL	Colorectal Cancer
CCS	Cervical Cancer Screening
	Cardiovascular Conditions
CBP	Controlling High Blood Pressure
	Diabetes
CDC_HBA8	HbA1c control (<8.0%)
CDC_Nephropathy	Medical attention for nephropathy
CDC_EyeExam	Diabetic Eye Exam
CDC_HBA_Testing	Diabetes: HBA1C Testing
	Overuse/Appropriateness
AAB	Avoidance of Antibiotic Treatment in Adults with Acute Bronchitis
	Access/Availability of Care
AAP	Adult Access to Preventive/Ambulatory Health Services (AAP)
	Risk Adjusted Utilization
PCR	Plan All-Cause Readmissions
	Behavioral Health
PHQ9	Preventative Care and Screening: Screening for Clinical Depression

PEDIATRIC PROGRAM METRICS

Metric Identifier	
	Prevention and Screening
CIS-8	Childhood Immunization Status - Combo 8
WCC	Weight Assessment and Counseling for Nutrition and Physical Activity for Children/Adolescents (BMI)
IMA-2	Immunizations for Adolescents (Combo 2)
	Respiratory Conditions
CWP	Appropriate Testing for Children With Pharyngitis
	Access/Availability of Care
CAP	Children and Adolescents' Access to Primary Care Practitioners Age 7-11
	Risk Adjusted Utilization
AWC	Adolescent Well-Care Visits
W34	Well-Child Visits in the Third, Fourth, Fifth and Sixth Years of Life
W15	Well-Child Visits in the First 15 Months of Life.

DESCRIPTION

Percent of women age 50-74 years old who had a mammogram screen during the measurement period or the year prior to the measurement period

FORMULA

Numerator

Number of women as defined in the denominator with one (or more) mammogram(s) any time on or between October 1 two years prior to the measurement year and December 31 of the measurement year.

Denominator

The eligible population.

ELIGIBLE POPULATION:

Product line (s)	Medicare Advantage, All Commercial (HMO, POS, Direct Access, PPO and Indemnity)
Age (s)	Women aged 52-74 years old as of December 31 st of the measurement year
Attribution Date	Members attributed to the Practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Continuous Enrollment	October 1 two years prior to the measurement year through December 31 of the measurement year.
Benefit(s)	Medical
Diagnosis/event	N/A
Exclusions:	Women who had a bilateral mastectomy, looking as far back as possible, that must have occurred by the last month of the measurement year, and for whom administrative data do not indicate that a mammogram was performed • A bilateral mastectomy code. • A unilateral mastectomy code with a bilateral modifier. Codes must be on the same claim • Two unilateral mastectomy codes on different dates of service. • A unilateral mastectomy code with a right side modifier and a unilateral mastectomy code with a left side modifier (may be on the same date of service).
Other	N/A

MEASUREMENT PERIOD

3 years

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data for exclusions. PRACTICE may refine the denominator to indicate clinical exclusion not found in claims data.

Numerator – supplied by Horizon BCBSNJ from claims data. PRACTICE may supplement and fill gaps with clinical information found in their medical charts. Documentation should indicate date of screening.

CLINICAL REFERENCE

HEDIS 2017, Volume 2, p.74

COMMENTS

This measure evaluates primary screening. Do not count biopsies, breast ultrasounds, MRIs or tomosynthesis (3D mammography), because they are not appropriate methods for primary breast cancer screening.

CODE SETS

Please see Value set codes for (BCS)

1. Codes to indicate Breast Cancer Screening

Breast Cancer Screening Mammography

2. Codes to identify Exclusions

Bilateral Mastectomy
Bilateral Modifier
Left Modifier
Right Modifier
Unilateral Mastectomy
Absence of Right Breast
Absence of Left Breast

DESCRIPTION

The percentage of women 21–64 years of age who were screened for cervical cancer using either of the following criteria:

- Women age 21–64 who had cervical cytology performed every 3 years.
- Women age 30–64 who had cervical cytology/human papillomavirus (HPV) co-testing performed every 5 years.

FORMULA

Numerator

The number of women who were screened for cervical cancer, as identified in steps 1 and 2 below.

Step 1 -Identify women 24–64 years of age as of December 31 of the measurement year who had cervical cytology during the measurement year or the two years prior to the measurement year.

Step 2 From the women who did not meet step 1 criteria, identify women 30–64 years of age as of December 31 of the measurement year who had cervical cytology and a human papillomavirus with service dates four or less days apart during the measurement year or the four years prior to the measurement year **and** who were 30 years or older on the date of both tests. For example, if the service date for cervical cytology was December 1 of the measurement year, then the HPV test must include a service date on or between November 27 and December 5 of the measurement year.

Step 3 Sum the events from steps 1 and 2 to obtain the rate.

Denominator

The eligible population.

ELIGIBLE POPULATION:

Product line (s)	All Commercial (HMO, POS, Direct Access, PPO and Indemnity)
Age (s)	Women 24–64 years as of December 31 of the measurement year.
Attribution Date	Members attributed to the Practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Continuous Enrollment	The measurement year and the two years prior to the measurement year.
Benefit(s)	Medical
Diagnosis/event	N/A
Exclusions:	Hysterectomy with no residual cervix, cervical agenesis or acquired

	absence of cervix any time during the member's history through December 31 of the measurement year
Other	N/A

MEASUREMENT PERIOD

3 years

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data for exclusions. PRACTICE may refine the denominator to indicate clinical exclusion not found in claims data.

Numerator – supplied by Horizon BCBSNJ from claims data. PRACTICE may supplement and fill gaps with clinical information found in their medical charts. Documentation should indicate date of screening.

CLINICAL REFERENCE

HEDIS 2017, Volume 2, p.76

COMMENTS

None

CODE SETS

Please see Value set codes for (CCS)

Codes to indicate Cervical Cancer Screening

Cervical Cytology
HPV
HPV Tests

Codes to identify Exclusions

Absence of Cervix

DESCRIPTION

The percentage of members 50–75 years of age who had an appropriate screening for colorectal cancer

FORMULA
Numerator

The number of members as defined in the denominator who had one or more screenings for colorectal cancer. Appropriate screenings are defined by any one of the following criteria:

- Fecal occult blood test during the measurement year.
- Flexible sigmoidoscopy during the measurement year or the four years prior to the measurement year
- Colonoscopy during the measurement year or the nine years prior to the measurement year
- CT colonography during the measurement year or the four years prior to the measurement year.
- FIT-DNA during the measurement year or the two years prior to the measurement year.

Denominator

The eligible population

ELIGIBLE POPULATION:

Product line (s)	Medicare Advantage, All Commercial (HMO, POS, Direct Access, PPO and Indemnity)
Age (s)	Age 51-75 years or older as of December 31st of the measurement year.
Attribution Date	Members attributed to the Practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Continuous Enrollment	The measurement year and the year prior to the measurement year.
Benefit(s)	Medical
Diagnosis/event	N/A
Exclusions:	Members with a diagnosis of colorectal cancer or total colectomy. Look for evidence of colorectal cancer or total colectomy as far back as possible in the member's history. Exclusionary evidence in the medical record must include a note indicating a diagnosis of colorectal cancer or total colectomy. The diagnosis must have occurred by last month of the measurement year.
Other	N/A

MEASUREMENT PERIOD

Dependent on test used to satisfy the numerator:

- FOBT – one (1) year during the measurement year.
- Flexible sigmoidoscopy - during the measurement year or the four (4) years prior to the measurement year.
- Colonoscopy - during the measurement year or the nine (9) years prior to the measurement year.
- CT colonography during the measurement year or the four (4) years prior to the measurement year.
- FIT-DNA during the measurement year or the two (2) years prior to the measurement year.

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data for exclusions. Medical Practice may refine the denominator to indicate clinical exclusion not found in claims data.

Numerator – supplied by Horizon BCBSNJ from claims data and clinical lab data. PRACTICE may supplement and fill gaps with clinical information found in their medical charts. Documentation should indicate date of screening.

CLINICAL REFERENCE

HEDIS 2017, Volume 2, p. 80

COMMENTS

FOBT tests performed in an office setting or performed on a sample collected via a digital rectal exam (DRE) do not meet criteria

CODE SETS

Please see Value set codes for (COL)

1. Codes to indicate Colorectal Screening

Colonoscopy value set code
FOBT value set code
Flexible sigmoidoscopy value set code
CT Colonography Value Set
FIT-DNA Value Set

2. Codes to indicate Exclusions

Exclusions Colorectal cancer value set code

Total colectomy value set code

FINAL

Controlling High Blood Pressure (CBP)

DESCRIPTION

The percentage of members 18–85 years of age who had a diagnosis of hypertension and whose blood pressure (BP) was adequately controlled during the measurement year based on the following criteria:

- Members 18–59 years of age whose BP was <140/90 mm Hg.
- Members 60–85 years of age with a diagnosis of diabetes whose BP was <140/90 mm Hg.
- Members 60–85 years of age without a diagnosis of diabetes whose BP was <150/90 mm Hg.

Note: A single rate is reported and is the sum of all three groups.

FORMULA

Numerator

Number of patients as defined in the denominator who whose most recent BP is adequately controlled during the measurement year. Identify the most recent BP reading noted during the measurement year. The reading must occur after the date on which the diagnosis of hypertension was made or confirmed.

Denominator

The eligible population.

ELIGIBLE POPULATION:

Product line (s)	Medicare Advantage, All Commercial (HMO, POS, Direct Access, PPO and Indemnity)
Age (s)	Age 18-85 years old as of December 31 st of the measurement year
Continuous Enrollment	The measurement year
Attribution Date	Members attributed to the Practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Benefit(s)	Medical
Diagnosis/event	A member is considered hypertensive if there is at least one outpatient encounter with a diagnosis of hypertension during the first six months of the measurement year.
Exclusions:	Exclude from the eligible population all members with evidence of end-stage renal disease or kidney transplant on or prior to December 31 of the measurement year. Documentation in the medical record must include a dated note indicating evidence of ESRD, kidney transplant or dialysis. Exclude from the eligible population all members with a diagnosis of pregnancy during the measurement year. Exclude from the eligible population all members who had a nonacute inpatient admission during the measurement year. To identify nonacute

	inpatient admissions: 1. Identify all acute and nonacute inpatient stays 2. Confirm the stay was for nonacute care based on the presence of a nonacute code on the claim. 3. Identify the discharge date for the stay. Exclude from the eligible population all members who had an admission to a nonacute inpatient setting any time during the measurement year.
Other	N/A

MEASUREMENT PERIOD

1 Year

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data. Medical Practice may refine the denominator to indicate clinical exclusion not found in claims data.

Numerator – supplied by Medical Practice with clinical information documented in their medical charts. Documentation in the medical record must indicate the date of the BP and both the systolic and diastolic results.

CLINICAL REFERENCE

HEDIS 2017, Volume 2 p.116

CODE SETS

Please see Value set codes for (CBP)

Codes to Identify Outpatient Visits

Outpatient CPT
Hypertension

Codes to Identify Exclusions

ESRD
ESRD Obsolete
Pregnancy
Inpatient Stay
Nonacute Inpatient Stay

DESCRIPTION

The percentage of members 18–75 years of age with diabetes (type 1 and type 2) who had each of the following:

- Hemoglobin A1c (HbA1c) testing.
- HbA1c control (<8.0%).
- Eye exam (retinal) performed.
- Medical attention for nephropathy.

Note each is measured separately as appropriate to the program

FORMULA

Numerator

Number of patients as defined in the denominator who had the following:

- Hemoglobin A1c (HbA1c) testing.
- HbA1c control (<8.0%).
- Eye exam (retinal) performed.
- Medical attention for nephropathy.

Note each is measured separately as appropriate to the program

Denominator

The eligible population

ELIGIBLE POPULATION

Product line (s)	Medicare Advantage, All Commercial (HMO, POS, Direct Access, PPO and Indemnity)
Age (s)	18–75 years as of December 31 st of the measurement period
Continuous Enrollment	The measurement year.
Attribution Date	Members attributed to the Practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Benefit(s)	Medical
Diagnosis/event	There are two ways to identify members with diabetes: by claim/encounter data and by pharmacy data. The organization must use both methods to identify the eligible population, but a member only needs to be identified by one method to be included in the measure. Members may be identified as having diabetes during the measurement year or the year prior to the measurement year.

	<i>Claim/encounter data.</i> Members who met any of the following criteria during the measurement year or the year prior to the measurement year (count services that occur over both years): • At least two outpatient visits, observation visits, ED visits or nonacute inpatient encounters on different dates of service, with a diagnosis of diabetes. Visit type need not be the same for the two visits. • At least one acute inpatient encounter with a diagnosis of diabetes. <i>Pharmacy data.</i> Members who were dispensed insulin or hypoglycemics/ antihyperglycemics on an ambulatory basis during the measurement year or the year prior to the measurement year
Exclusions	Members with a diagnosis gestational or steroid-induced diabetes. Diagnosis may occur during the measurement year or the year before the measurement year, but must have occurred by December 31 of the measurement year.

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims & pharmacy data. Medical Practice may refine the denominator to indicate clinical exclusion not found in administrative data or may exclude members who may not be clinical diabetics. PRACTICE may add members from attribution who may be diabetics as appropriate.

Numerator – supplied by Horizon BCBSNJ from claims and clinical lab data. Medical Practice may supplement and fill gaps with clinical information found in their medical charts. Documentation allowed for numerator is as followed:

HbA1c Testing - At a minimum, documentation in the medical record must include a note indicating the date when the HbA1c test was performed and the result or finding. Count notation of the following in the medical record:

- A1c.
- HbA1c.
- Hemoglobin A1c.
- Glycohemoglobin A1c.
- HgbA1c.

HbA1c Poor Control >9% - At a minimum, documentation in the medical record must include a note indicating the date when the HbA1c test was performed and the result. The member is numerator compliant if the result for the most recent HbA1c level during the measurement year is >9.0% or is missing, or if an HbA1c test was not done during the measurement year. The member is not numerator compliant if the most recent HbA1c level during the measurement year is ≤9.0%. Ranges and thresholds do not meet criteria for this indicator. A distinct numeric result is required for numerator compliance.

HbA1c Control <8% - At a minimum, documentation in the medical record must include a note indicating the date when the HbA1c test was performed and the result. The member is numerator compliant if the most recent HbA1c level during the measurement year is <8.0%. The member is not numerator compliant if the result for the most recent HbA1c level during the measurement year is ≥8.0% or is missing, or if an HbA1c test was not performed during the measurement year. Ranges and thresholds do not meet criteria for this indicator. A distinct numeric result is required for numerator compliance.

HbA1c Control <7% for a Selected Population - At a minimum, documentation in the medical record must include a note indicating the date when the HbA1c test was performed and the result. The member is numerator compliant if the most recent HbA1c level during the measurement year is <7.0%. The member is not numerator compliant if the result for the

most recent HbA1c level during the measurement year is $\geq 7.0\%$ or is missing, or if an HbA1c test was not performed during the measurement year. Ranges and thresholds do not meet criteria for this indicator. A distinct numeric result is required for numerator compliance.

Eye Exam - At a minimum, documentation in the medical record must include one of the following:

- A note or letter prepared by an ophthalmologist, optometrist, PCP or other health care professional indicating that an ophthalmoscopic exam was completed by an eye care professional (optometrist or ophthalmologist), the date when the procedure was performed and the results.
- A chart or photograph of retinal abnormalities indicating the date when the fundus photography was performed and evidence that an eye care professional (optometrist or ophthalmologist) reviewed the results. Alternatively, results may be read by a qualified reading center that operates under the direction of a medical director who is a retinal specialist.
- Documentation of a negative retinal or dilated exam by an eye care professional (optometrist or ophthalmologist) in the year prior to the measurement year, where results indicate retinopathy was not present (e.g., documentation of normal findings for a dilated or retinal eye exam performed by an eye care professional (optometrist or ophthalmologist) meets criteria).

Medical Attention for Nephropathy - Any of the following meet criteria for a nephropathy screening or monitoring test or evidence of nephropathy:

- A urine test for albumin or protein. At a minimum, documentation must include a note indicating the date when a urine test was performed, and the result or finding. Any of the following meet the criteria:
 - 24-hour urine for albumin or protein.
 - Timed urine for albumin or protein.
 - Spot urine for albumin or protein.
 - Urine for albumin/creatinine ratio.
 - 24-hour urine for total protein.
 - Random urine for protein/creatinine ratio.
- Documentation of a visit to a nephrologist.
- Documentation of a renal transplant.
- Documentation of medical attention for any of the following (no restriction on provider type):
 - Diabetic nephropathy.
 - ESRD.
 - Chronic renal failure (CRF).
 - Chronic kidney disease (CKD).
 - Renal insufficiency.
 - Proteinuria.
 - Albuminuria.
 - Renal dysfunction.
 - Acute renal failure (ARF).
 - Dialysis, hemodialysis or peritoneal dialysis.
- Evidence of ACE inhibitor/ARB therapy. Documentation in the medical record must include, at minimum, a note indicating that the member received an ambulatory prescription for ACE inhibitors/ARBs in the measurement year.

BP Control $< 140/90$ mm Hg - The organization should use the medical record from which it abstracts data for the other CDC indicators. If the organization does not abstract for other indicators, it should use the medical record of the provider that manages the member's diabetes. If that medical record does not contain a BP, the organization may use the medical record of another PCP or specialist from whom the member receives care.

To determine if BP is adequately controlled, the organization must identify the representative BP following the steps below.
Step 1 Identify the most recent BP reading noted during the measurement year. Do not include BP readings that meet the following criteria:

- Taken during an acute inpatient stay or an ED visit.
- Taken during an outpatient visit which was for the sole purpose of having a diagnostic test or surgical procedure performed (e.g., sigmoidoscopy, removal of a mole).
- Obtained the same day as a major diagnostic or surgical procedure (e.g., EKG/ECG, stress test, administration of IV contrast for a radiology procedure, endoscopy).
- Reported by or taken by the member.

Step 2 Identify the lowest systolic and lowest diastolic BP reading from the most recent BP notation in the medical record. If there are multiple BPs recorded for a single date, use the lowest systolic and lowest diastolic BP on that date as the representative BP. The systolic and diastolic results do not need to be from the same reading when multiple readings are recorded for a single date. The member is not numerator compliant if the BP does not meet the specified threshold or is missing, or if there is no BP reading during the measurement year or if the reading is incomplete (i.e., the systolic or diastolic level is missing).

CLINICAL REFERENCE

HEDIS 2017 Volume 2, p. 132

CODES & CODE SETS

Acute Inpatient
Blindness
CABG
Chronic Heart Failure
CKD Stage 4
Dementia
Diabetes
Diabetes Exclusions
Diabetic Retinal Screening
Diabetic Retinal Screening Negative
Diabetic Retinal Screening With Eye Care Professional
Diastolic 80-89
Diastolic Greater Than/Equal To 90
Diastolic Less Than 80
ED
ESRD
ESRD Obsolete
Frontotemporal Dementia
HbA1c Level 7.0-9.0
HbA1c Level Greater Than 9.0
HbA1c Level Less Than 7.0
HbA1c Tests
Inpatient Stay
IVD
Kidney Transplant
Lower Extremity Amputation
MI
Nephropathy Treatment
Nonacute Inpatient
Observation
Outpatient
PCI
Systolic Greater Than/Equal To 140
Systolic Less Than 140
Thoracic Aortic Aneurysm
Urine Protein Tests

Prescriptions to Identify Members with Diabetes

Table CDC-A: Prescriptions to Identify Members With Diabetes

Description	Prescription			
Alpha-glucosidase inhibitors	• Acarbose	• Miglitol		
Amylin analogs	• Pramlintide			
Antidiabetic combinations	• Alogliptin-metformin • Alogliptin-pioglitazone • Canagliflozin-metformin • Glimepiride-pioglitazone • Glimepiride-rosiglitazone • Glipizide-metformin	• Glyburide-metformin • Linagliptin-metformin • Metformin-pioglitazone • Metformin-repaglinide • Metformin-rosiglitazone • Metformin-saxagliptin	• Metformin-sitagliptin • Sitagliptin-simvastatin	
Insulin	• Insulin aspart • Insulin aspart-insulin aspart protamine • Insulin detemir • Insulin glargine • Insulin glulisine	• Insulin isophane human • Insulin isophane-insulin regular • Insulin lispro • Insulin lispro-insulin lispro protamine • Insulin regular human		
Meglitinides	• Nateglinide	• Repaglinide		
Glucagon-like peptide-1 (GLP1) agonists	• Exenatide	• Liraglutide	• Albiglutide	
Sodium glucose cotransporter 2 (SGLT2) inhibitor	• Canagliflozin	• Dapagliflozin	• Empagliflozin	
Sulfonylureas	• Chlorpropamide • Glimepiride	• Glipizide • Glyburide	• Tolazamide • Tolbutamide	
Thiazolidinediones	• Pioglitazone	• Rosiglitazone		
Dipeptidyl peptidase-4 (DDP-4) inhibitors	• Alogliptin • Linagliptin	• Saxagliptin • Sitagliptin		

Note: Glucophage/metformin as a solo agent is not included because it is used to treat conditions other than diabetes; members with diabetes on these medications are identified through diagnosis codes only. NCQA will post a complete list of medications and NDC codes to www.ncqa.org by November 2, 2015.

ACE Inhibitors/ARBs

Description	Prescription					
Angiotensin converting enzyme inhibitors	• Benazepril • Captopril	• Enalapril • Fosinopril	• Lisinopril • Moexipril	• Perindopril • Quinapril	• Ramipril • Trandolapril	
Angiotensin II inhibitors	• Azilsartan • Candesartan	• Eprosartan • Irbesartan	• Losartan • Olmesartan	• Telmisartan • Valsartan		
Antihypertensive combinations	• Aliskiren-valsartan • Amlodipine-benazepril • Amlodipine-hydrochlorothiazide-valsartan • Amlodipine-hydrochlorothiazide-olmesartan • Amlodipine-olmesartan	• Azilsartan-chlorthalidone • Benazepril-hydrochlorothiazide • Candesartan-hydrochlorothiazide • Captopril-hydrochlorothiazide • Enalapril-hydrochlorothiazide • Eprosartan-hydrochlorothiazide • Fosinopril-hydrochlorothiazide	• Hydrochlorothiazide-lisinopril • Hydrochlorothiazide-losartan • Hydrochlorothiazide-moexipril • Hydrochlorothiazide-olmesartan • Hydrochlorothiazide-quinapril • Hydrochlorothiazide-telmisartan • Hydrochlorothiazide-valsartan			

	• Amlodipine-telmisartan • Amlodipine-valsartan	• Hydrochlorothiazide-irbesartan	• Trandolapril-verapamil
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Note: NCQA will post a comprehensive list of medications and NDC codes to www.ncqa.org by November 1, 2016.

FINAL

Avoidance of Antibiotic Treatment in Adults With Acute Bronchitis (AAB)

DESCRIPTION

The percentage of adults 18–64 years of age with a diagnosis of acute bronchitis who were not dispensed an antibiotic prescription.

FORMULA

The measure is reported as an inverted rate $[1 - (\text{numerator}/\text{eligible population})]$. A higher rate indicates appropriate treatment of adults with acute bronchitis (i.e., the proportion for whom antibiotics were *not* prescribed).

Numerator

Members who identified in the denominator dispensed prescription for antibiotic medication on or three days after the IESD.

Denominator

The eligible population.

ELIGIBLE POPULATION:

Product line (s)	Commercial
Age (s)	Adults 18 years as of January 1 of the year prior to the measurement year to 64 years as of December 31 of the measurement year.
Attribution Date	Members attributed to the Practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Continuous Enrollment	One year prior to the Episode Date through seven days after the Episode Date (373 total days).
Benefit(s)	Medical & Pharmacy
Diagnosis/event	Event/diagnosis Outpatient or ED visit during the Intake Period with any diagnosis of acute bronchitis. Follow the steps below to identify the eligible population: Step 1 Identify all members in the specified age range who had an outpatient visit, an observation visit or an ED visit during the Intake Period, with a diagnosis of acute bronchitis). Do not include ED visits or observation visits that result in an inpatient stay. An ED visit or observation visit results in an inpatient stay when the ED/observation date of service and the admission date for the inpatient stay are one calendar day apart or less. Step 2 Determine all acute bronchitis Episode Dates. For each member identified in

	Step 1, determine all outpatient or ED claims/encounters with a diagnosis of acute bronchitis. Step 3 Test for Negative Comorbid Condition History. Exclude Episode Dates when the member had a claim/encounter with any diagnosis for a comorbid condition during the 12 months prior to or on the Episode Date. A code from any of the following meets criteria for a comorbid condition: • HIV Value Set. • Malignant Neoplasms Value Set. • Emphysema Value Set. • COPD Value Set. • Cystic Fibrosis Value Set. • Comorbid Conditions Value Set. Step 4 Test for Negative Medication History. Exclude Episode Dates where a new or refill prescription for an antibiotic medication (Table AAB-D) was filled 30 days prior to the Episode Date or was active on the Episode Date. Step 5 Test for Negative Competing Diagnosis. Exclude Episode Dates where during the period 30 days prior to the Episode Date through 7 days after the Episode Date (38 total days) the member had a claim/encounter with any competing diagnosis. A code from either of the following meets criteria for a competing diagnosis: • Pharyngitis Value Set. • Competing Diagnosis Value Set. Step 6 Calculate continuous enrollment. The member must be continuously enrolled with no more than one gap in coverage from 365 days (1 year) prior to the Episode Date through 7 days after the Episode Date (373 total days). Step 7 Select the IESD. This measure examines the earliest eligible episode per member.
Exclusions:	N/A
Other	N/A

MEASUREMENT PERIOD

1 Year

DATA SOURCE

Denominator – supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data for exclusions. Rx data supplied from Prime Therapeutics.

Numerator – supplied by Horizon BCBSNJ from claims and clinical lab data. Rx data supplied from Prime Therapeutics.

CLINICAL REFERENCE

CODES

Outpatient Value Set

Observation Value Set

ED Value Set

Acute Bronchitis Value Set

HIV Value Set.

HIV Type 2 Value Set

Disorders of the Immune System Value Set

Malignant Neoplasms Value Set.

Emphysema Value Set.

COPD Value Set.

Cystic Fibrosis Value Set.

Comorbid Conditions Value Set.

Pharyngitis Value Set.

Competing Diagnosis Value Set.

Antibiotic Medications

Description	Prescription		
Aminoglycosides	• Amikacin • Gentamicin	• Kanamycin • Streptomycin	• Tobramycin
Aminopenicillins	• Amoxicillin	• Ampicillin	
Antipseudomonal penicillins	• Piperacillin		
Beta-lactamase inhibitors	• Amoxicillin-clavulanate • Ampicillin-sulbactam	• Piperacillin-tazobactam	• Ticarcillin-clavulanate
First-generation cephalosporins	• Cefadroxil	• Cefazolin	• Cephalexin
Fourth-generation cephalosporins	• Cefepime		
Ketolides	• Telithromycin		
Lincomycin derivatives	• Clindamycin	• Lincomycin	
Macrolides	• Azithromycin • Clarithromycin	• Erythromycin • Erythromycin ethylsuccinate	• Erythromycin lactobionate • Erythromycin stearate
Miscellaneous antibiotics	• Aztreonam • Chloramphenicol • Dalfopristin-quinupristin	• Daptomycin • Erythromycin-sulfisoxazole • Linezolid	• Metronidazole • Vancomycin
Natural penicillins	• Penicillin G benzathine-procaine • Penicillin G potassium	• Penicillin G procaine • Penicillin G sodium	• Penicillin V potassium • Penicillin G benzathine
Penicillinase resistant penicillins	• Dicloxacillin	• Nafcillin	• Oxacillin
Quinolones	• Ciprofloxacin • Gemifloxacin	• Levofloxacin • Moxifloxacin	• Norfloxacin • Ofloxacin
Rifamycin derivatives	• Rifampin		
Second generation cephalosporin	• Cefaclor • Cefotetan	• Cefoxitin • Cefprozil	• Cefuroxime
Sulfonamides	• Sulfadiazine	• Sulfamethoxazole-trimethoprim	
Tetracyclines	• Doxycycline	• Minocycline	• Tetracycline

Third generation cephalosporins	• Cefdinir • Cefditoren • Cefixime	• Cefotaxime • Cefpodoxime • Ceftazidime	• Ceftibuten • Ceftriaxone
Urinary anti-infectives	• Fosfomycin • Nitrofurantoin • Nitrofurantoin macrocrystals	• Nitrofurantoin macrocrystals-monohydrate • Trimethoprim	

Note: NCQA will post a comprehensive list of medications and NDC codes to www.ncqa.org by November 2, 2016.

FINAL

Adult Access to Preventive/Ambulatory Health Services (AAP)

DESCRIPTION

The percentage of Commercial members 20 years and older who had an ambulatory or preventive care visit during the measurement year or the two years prior to the measurement year.

FORMULA

Numerator

One or more ambulatory or preventive care visits during the measurement year or the two years prior to the measurement year.

Denominator

The eligible population.

ELIGIBLE POPULATION:

Product line (s)	All Commercial (HMO, POS, Direct Access, PPO and Indemnity)
Age (s)	20–65 years and older as of December 31 st of the measurement year.
Attribution Date	Members attributed to the Practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Continuous Enrollment	The measurement year and the two years prior to the measurement year.
Benefit(s)	Medical
Diagnosis/event	N/A
Exclusions:	N/A
Other	N/A

MEASUREMENT PERIOD

3 Year

DATA SOURCE

Denominator – supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data for exclusions.

Numerator – supplied by Horizon BCBSNJ from claims and clinical lab data.

CLINICAL REFERENCE

COMMENTS

CODE SETS

Please see Value set codes for (AAP)

- Ambulatory Visit
- Other Ambulatory Visit

FINAL

DESCRIPTION

For members 18-64 years of age and older, the number of acute inpatient stays during the measurement year that were followed by an unplanned acute readmission for any diagnosis within 30 days and the predicted probability of an acute readmission. Data are reported in the following categories:

1. Count of Index Hospital Stays (IHS) (denominator).
2. Count of 30-Day Readmissions (numerator).
3. Average Adjusted Probability of Readmission.

FORMULA

Numerator

Number of patients as defined in the denominator who had at least one acute readmission for any diagnosis within 30 days of the Index Discharge Date.

Denominator

The eligible population

ELIGIBLE POPULATION

Product line (s)	Commercial,
Age (s)	Ages 18–64 as of the Index Discharge Date
Continuous Enrollment	365 days prior to the Index Discharge Date through 30 days after the Index Discharge Date.
Attribution Date	Members attributed to the practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Benefit(s)	Medical
Diagnosis/event	An acute inpatient discharge on or between January 1 and December 1 of the measurement year. The denominator for this measure is based on discharges, not members. Include all acute inpatient discharges for members who had one or more discharges on or between January 1 and December 1 of the measurement year. Follow the steps below to identify acute inpatient stays. Step 1: Identify all acute inpatient discharges on or between January 1 and December 1 of the measurement year. To identify acute inpatient discharges: 1. Identify all acute and nonacute inpatient stays 2. Exclude nonacute inpatient stays 3. Identify the discharge date for the stay. The measure includes acute discharges from any type of facility

	(including behavioral healthcare facilities). Step 2 Acute-to-acute transfers: Keep the original admission date as the Index Admission Date, but use the transfer's discharge date as the Index Discharge Date. Organizations must identify "transfers" using their own methods and then confirm the acute inpatient care setting using the process in step 1. Step 3 Exclude hospital stays where the Index Admission Date is the same as the Index Discharge Date. Step 4: Required exclusions Exclude hospital stays for the following reasons: • The member died during the stay. • A principal diagnosis of pregnancy • A principal diagnosis of a condition originating in the perinatal period Note: For hospital stays where there was an acute-to-acute transfer (identified in step 2), use both the original stay and the transfer stay to identify exclusions in this step. Step 5: Required exclusions For all acute inpatient discharges identified using steps 1–4, determine if there was a planned hospital stay within 30 days. To identify planned hospital stays, identify all acute inpatient discharges on or between January 1 and December 31 of the measurement year: 1. Identify all acute and nonacute inpatient stays 2. Exclude nonacute inpatient stays 3. Identify the admission date for the stay. 4. Exclude any hospital stay as an Index Hospital Stay if the admission date of the first stay within 30 days meets any of the following criteria: – A principal diagnosis of maintenance chemotherapy – A principal diagnosis of rehabilitation – An organ transplant – A potentially planned procedure without a principal acute diagnosis Note: For hospital stays where there was an acute-to-acute transfer (identified in step 2), use only the original stay to identify planned hospital stays in this step (i.e., do not use diagnoses and procedures from the transfer stay).
Exclusions	See Above

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data & Pharmacy data.

Numerator – supplied by Horizon BCBSNJ claims data.

CLINICAL REFERENCE

HEDIS 2017 Volume 2, p. 345

CODES

Inpatient Stay Value Set

Nonacute Inpatient Stay Value Set

Pregnancy Value Set

Perinatal Conditions Value Set

Chemotherapy Value Set

Rehabilitation Value Set

Kidney Transplant Value Set, Bone Marrow Transplant Value Set, Organ Transplant Other Than Kidney Value Set

Potentially Planned Procedures Value Set

Acute Condition Value Set

FINAL

Preventative Care and Screening: Screening for Clinical Depression (PQ9)

DESCRIPTION

Percentage of patients aged 18 years and older screened for clinical depression on the date of the encounter using an age appropriate standardized depression screening tool. Examples of depression screening tools include but are not limited to: Patient Health Questionnaire (PHQ-9), Beck Depression Inventory (BDI or BDI-II), Center for Epidemiologic Studies Depression Scale (CES-D), Depression Scale (DEPS), Duke Anxiety-Depression Scale (DADS), Geriatric Depression Scale (GDS), Cornell Scale Screening, and PRIME MD-PHQ2.

FORMULA

Numerator

Number of members as defined in the denominator who were screened for clinical depression on the date of the encounter.

Denominator

The eligible population

ELIGIBLE POPULATION

Product line (s)	All Commercial & Medicare
Age (s)	18 years and older as of the last day of the measurement year.
Continuous Enrollment	N/A
Attribution Date	Members attributed to the practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Benefit(s)	Medical
Diagnosis/event	All members who had an outpatient visit with attributed practice during the measurement year.
Exclusions	None

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data.

Numerator – supplied by Horizon BCBSNJ claims data and Medical Practice may supplement and fill gaps with clinical information found in their Disease Registry. The name of the age appropriate standardized depression screening tool utilized must be documented in the medical record.

CLINICAL REFERENCE

2015 American Medical Association
Measure #134 (NQF 0418): Preventive Care and Screening: Screening for Clinical Depression and

CODES

Codes to patient encounter during the reporting period (CPT or HCPCS): 90791, 90792, 90832, 90834, 90837, 90839, 92625, 96116, 96118, 96150, 96151, 97003, 99201, 99202, 99203, 99204, 99205, 99212, 99213, 99214, 99215, G0101, G0402, G0438, G0439, G0444

Codes to identify Depression Screening (CPT)
96127

FINAL

DESCRIPTION

The percentage of children 2 years of age who had four diphtheria, tetanus and acellular pertussis (DTaP); three polio (IPV); one measles, mumps and rubella (MMR); three haemophilus influenza type B (HiB); three hepatitis B (HepB), one chicken pox (VZV); four pneumococcal conjugate (PCV); one hepatitis A (HepA); and two influenza (flu) vaccines by their second birthday.

FORMULA

Numerator

Number of children as defined in the denominator who received four DTaP, three IPV, one MMR (or documented history of the illness), three HiB, three hepatitis B vaccinations (or documented history of the illness) four PCV, one VZV (or documented history of the illness) and two flu vaccines on or before the child's second birthday

Denominator

The eligible population.

ELIGIBLE POPULATION:

Product line (s)	Commercial
Age (s)	Children who turn 2 years of age during the measurement year.
Attribution Date	Members attributed to the practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Continuous Enrollment	12 months prior to the child's second birthday
Benefit(s)	Medical
Diagnosis/event	None
Exclusions:	Exclude children who had a contraindication for a specific vaccine
Other	N/A

MEASUREMENT PERIOD

1 year

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data for exclusions. Medical Practice may refine the denominator to indicate clinical exclusion not found in claims data.

Numerator – supplied by Horizon BCBSNJ from claims data.

CLINICAL REFERENCE

HEDIS 2017, Volume 2, p. 62

COMMENTS

CODE SETS

Please see Value set codes for (CIS-8)

- ✓ DTaP Vaccine Administered Value Set
- ✓ Inactivated Polio Vaccine (IPV) Administered Value Set
- ✓ Measles, Mumps and Rubella (MMR) Vaccine Administered Value Set
- ✓ Measles/Rubella Vaccine Administered Value Set
- ✓ Mumps Vaccine Administered Value Set
- ✓ Measles Value Set
- ✓ Mumps Value Set
- ✓ Rubella Value Set
- ✓ Haemophilus Influenzae Type B (HiB) Vaccine Administered Value Set
- ✓ Hepatitis B Vaccine Administered Value Set
- ✓ Hepatitis B Value Set
- ✓ Varicella Zoster (VZV) Vaccine Administered Value Set
- ✓ Varicella Zoster Value Set
- ✓ Pneumococcal Conjugate Vaccine Administered Value Set
- ✓ Influenza Vaccine Administered Value Set

Excusions

- ✓ Anaphylactic Reaction Due To Vaccination Value Set



Weight Assessment and Counseling for Nutrition and Physical Activity for Children/Adolescents (WCC)

DESCRIPTION

The percentage of attributed members 3–17 years of age who had an outpatient visit with a PCP or OB/GYN and who had evidence of the following during the measurement year:

- BMI percentile documentation*
- Counseling for nutrition
- Counseling for physical activity

*Because BMI norms for youth vary with age and gender, this measure evaluates whether BMI percentile is assessed rather than an absolute BMI value.

FORMULA

Numerator

Number of members as defined in the denominator who who had evidence of BMI percentile documentation, counseling for nutrition or physicians activity during the measurement year.

Denominator

The eligible population.

ELIGIBLE POPULATION:

Product line (s)	Commercial
Age (s)	Age 3–17 years of age as of the last day of the measurement year.
Attribution Date	Members attributed to the practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Continuous Enrollment	The measurement year
Benefit(s)	Medical
Diagnosis/event	An outpatient visit (Outpatient Value Set) with a PCP or an OB/GYN during the measurement year.
Exclusions:	Members who have a diagnosis of pregnancy (Pregnancy Value Set) during the measurement year. The denominator for all rates must be the same. An organization that excludes these members must do so for all rates.
Other	N/A

MEASUREMENT PERIOD

1 year

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data for exclusions. Medical Practice may refine the denominator to indicate clinical exclusion not found in claims data.

Numerator – supplied by Horizon BCBSNJ from claims data.

CLINICAL REFERENCE

HEDIS 2017, Volume 2, p. 57

COMMENTS

CODE SETS

Please see Value set codes for (WWC)

- ✓ Outpatient Value Set
- ✓ BMI Percentile Value Set
- ✓ BMI Value Set
- ✓ Nutrition Counseling Value Set
- ✓ Physical Activity Counseling Value Set
- ✓ Pregnancy Value Set

DESCRIPTION

The percentage of adolescents 13 years of age who had one dose of meningococcal conjugate vaccine, one tetanus, diphtheria toxoids and acellular pertussis (Tdap) vaccine and three doses of the human papillomavirus (HPV) vaccine by their 13th birthday.

FORMULA

Numerator

Number of children as defined in the denominator who received one dose of meningococcal vaccine with a date of service on or between the member's 11th and 13th birthdays; and at least one tetanus, diphtheria toxoids and acellular pertussis (Tdap) vaccine with a date of service on or between the member's 10th and 13th birthday; and At least three HPV, with different dates of service on or between the member's 9th and 13th birthdays.

Denominator

The eligible population.

ELIGIBLE POPULATION:

Product line (s)	Commercial
Age (s)	Adolescents who turn 13 years of age during the measurement year.
Attribution Date	Members attributed to the practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Continuous Enrollment	12 months prior to the member's 13th birthday.
Benefit(s)	Medical
Diagnosis/event	None
Exclusions:	Exclude adolescents who had a contraindication for a specific vaccine from the denominator for all antigen rates and the combination rate. The denominator for all rates must be the same. Contraindicated adolescents may be excluded only if administrative data do not indicate that the contraindicated immunization was rendered. The exclusion must have occurred by the member's 13th birthday.
Other	N/A

MEASUREMENT PERIOD

1 year

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data for exclusions. Medical Practice may refine the denominator to indicate clinical exclusion not found in claims data.

Numerator – supplied by Horizon BCBSNJ from claims data.

CLINICAL REFERENCE

HEDIS 2017, Volume 2, p. 68

COMMENTS

CODE SETS

Please see Value set codes for (IMA)

Meningococcal Vaccine Administered Value Set

Tdap Vaccine Administered Value Set

Td Vaccine Administered Value Set

Tetanus Vaccine Administered Value Set

Diphtheria Vaccine Administered Value Set

HPV Vaccine Administered Value Set

Anaphylactic Reaction Due To Vaccination Value Set

Appropriate Testing for Children With Pharyngitis (CWP)

DESCRIPTION

The percentage of children 2–18 years of age, who were diagnosed with pharyngitis, dispensed an antibiotic and received a group A streptococcus (strep) test for the episode. A higher rate represents better performance (i.e., appropriate testing).

FORMULA

Numerator

A group A streptococcus test in the seven-day period from three days prior to the IESD (Index Episode Start Date) through three days after the IESD.

Denominator

The eligible population.

ELIGIBLE POPULATION:

Product line (s)	Commercial
Age (s)	Children 2 years as of July 1 of the year prior to the measurement year to 18 years as of June 30 of the measurement year
Attribution Date	Members attributed to the practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Continuous Enrollment	30 days prior to the Episode Date through 3 days after the Episode Date (inclusive).
Benefit(s)	Medical and pharmacy
Diagnosis/event	Outpatient or ED visit with only a diagnosis of pharyngitis and a dispensed antibiotic for that episode of care during the Intake Period. Follow the steps below to identify the eligible population. <u>Step 1:</u> Identify all members who had an outpatient or ED visit with only a diagnosis of pharyngitis during the Intake Period. Exclude claims/encounters with more than one diagnosis. <u>Step 2:</u> Determine all pharyngitis Episode Dates. For each member identified in step 1, determine all outpatient or ED claims/encounters with only a diagnosis of pharyngitis. <u>Step 3:</u> Determine if antibiotics were dispensed for any of the Episode Dates. For each Episode Date with a qualifying diagnosis, determine if antibiotics were dispensed on or up to three days after. Exclude Episode Dates if the member did not receive antibiotics on or three days after the Episode Date. <u>Step 4:</u> Test for Negative Medication History. Exclude Episode Dates where a new or refill prescription for an antibiotic medication was filled 30 days prior to the Episode Date or where

	a prescription filled more than 30 days prior to the Episode Date was active on the Episode Date. <u>Step 5:</u> Calculate continuous enrollment. The member must be continuously enrolled without a gap in coverage from 30 days prior to the Episode Date through 3 days after the Episode Date. <u>Step 6:</u> Select the IESD. This measure examines the earliest eligible episode per member.
Exclusions:	None
Other	N/A

MEASUREMENT PERIOD

1 year

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data for exclusions.

Numerator – supplied by Horizon BCBSNJ from claims data.

CLINICAL REFERENCE

HEDIS 2017, Volume 2, p. 94

CODE SETS

Outpatient Value Set
 Observation Value Set
 ED Value Set
 Pharyngitis Value Set

Antibiotic Medications

Description	Prescription	
Aminopenicillins	Amoxicillin	Ampicillin
Beta-lactamase inhibitors	Amoxicillin-clavulanate	
First generation cephalosporins	Cefadroxil	Cephalexin
	Cefazolin	
Folate antagonist	Trimethoprim	
Lincomycin derivatives	Clindamycin	
Macrolides	Azithromycin	Erythromycin ethylsuccinate
	Clarithromycin	Erythromycin lactobionate
	Erythromycin	Erythromycin stearate
Miscellaneous antibiotics	Erythromycin-sulfisoxazole	
Natural penicillins	Penicillin G potassium	Penicillin V potassium
	Penicillin G sodium	

Penicillinase-resistant penicillins	Dicloxacillin	
Quinolones	Ciprofloxacin	Moxifloxacin
	Levofloxacin	Ofloxacin
Second generation cephalosporins	Cefaclor	Cefuroxime
	Cefprozil	
Sulfonamides	Sulfamethoxazole-trimethoprim	
Tetracyclines	Doxycycline	Tetracycline
	Minocycline	
Third generation cephalosporins	Cefdinir	Ceftibuten
	Cefixime	Cefditoren
	Cefpodoxime	Ceftriaxone

Note: NCQA will post a comprehensive list of medications and NDC codes to www.ncqa.org by November 1, 2016.

Children and Adolescents' Access to Primary Care Practitioners (CAP) - Age Children 7-11

DESCRIPTION

The percentage of members 7-11 years of age who had a visit with a primary care physician (PCP) during the measurement year or the year prior to the measurement year.

FORMULA

Numerator

Number of patients as defined in the denominator who had a one or more visits with a PCP during the measurement year or the year prior to the measurement year.

Denominator

The eligible population

ELIGIBLE POPULATION

Product line (s)	Commercial
Age (s)	7-11 years as of December 31 of the measurement year.
Continuous Enrollment	The measurement year and the year prior to the measurement year.
Attribution Date	Members attributed to the practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Benefit(s)	Medical
Diagnosis/event	None
Exclusions	None

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data & Pharmacy data.

Numerator – supplied by Horizon BCBSNJ claims data.

CLINICAL REFERENCE

HEDIS 2017 Volume 2, p. 253

CODES

Ambulatory Visits Value Set

DESCRIPTION

The percentage of enrolled members 12–18 years of age who had at least one comprehensive well-care visit with a PCP or an OB/GYN practitioner during the measurement year.

FORMULA

Numerator

At least one comprehensive well-care visit with a PCP or an OB/GYN practitioner during the measurement year. The practitioner does not have to be the practitioner assigned to the member.

Denominator

The eligible population

ELIGIBLE POPULATION

Product line (s)	Commercial
Age (s)	12-18 years as of December 31 of the measurement year.
Continuous Enrollment	The measurement year.
Attribution Date	Members attributed to the practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Benefit(s)	Medical
Diagnosis/event	None
Exclusions	None

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data & Pharmacy data.

Numerator – supplied by Horizon BCBSNJ claims data.

CLINICAL REFERENCE

HEDIS 2017 Volume 2, p. 299

CODES

Well-Care Value Set

Well-Child Visits in the Third, Fourth, Fifth and Sixth Years of Life (W34)

DESCRIPTION

The percentage of members 3–6 years of age who had one or more well-child visits with a PCP during the measurement year.

FORMULA

Numerator

Number of members identify in the eligible population with at least one well-child visit with a PCP during the measurement year.

Denominator

The eligible population

ELIGIBLE POPULATION

Product line (s)	Commercial
Age (s)	3–6 years as of December 31 of the measurement year.
Continuous Enrollment	The measurement year
Attribution Date	Members attributed to the practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Benefit(s)	Medical
Diagnosis/event	None
Exclusions	None

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data.

Numerator – supplied by Horizon BCBSNJ claims data.

CLINICAL REFERENCE

HEDIS 2017 Volume 2, p. 296

COMMENTS

Services specific to the assessment or treatment of an acute or chronic condition do not count toward this measure.

CODES

Well-Care Value Set

Well-Child Visits in the First 15 Months of Life (W15)

DESCRIPTION

The percentage of members who turned 15 months old during the measurement year and who had the following number of well-child visits with a PCP during their first 15 months of life:

- Six or more well-child visits.

FORMULA

Numerator

Number of members identify in the eligible population with at least one well-child visit with a PCP during the measurement year.

Denominator

The eligible population

ELIGIBLE POPULATION

Product line (s)	Commercial
Age (s)	15 months old during the measurement year
Continuous Enrollment	31 days–15 months of age
Attribution Date	Members attributed to the practice as of the last day of the sixth month of the measurement period and enrolled at the last day of the measurement year.
Benefit(s)	Medical
Diagnosis/event	None
Exclusions	None

DATA SOURCE

Denominator - supplied by Horizon BCBSNJ, based on enrollment/attribution (see above) and claims data.

Numerator – supplied by Horizon BCBSNJ claims data.

CLINICAL REFERENCE

HEDIS 2017 Volume 2, p. 293

COMMENTS

Services specific to the assessment or treatment of an acute or chronic condition do not count toward this measure.

CODES

Well-Care Value Set